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Confusion, Dementia, and Alzheimer's Disease



1. Discuss confusion and delirium

Define the following terms:

confusion

the inability to think logically and clearly.

delirium

a state of severe confusion that occurs suddenly and is usually temporary.

1. Discuss confusion and delirium

Remember these points about confusion:

- Interferes with ability to make decisions
- Personality may change
- Anger, depression, and irritability are other signs
- Can be temporary or permanent

1. Discuss confusion and delirium

Causes of confusion include the following:

- Urinary tract infection (UTI)
- Low blood sugar
- Head trauma or injury
- Dehydration
- Nutritional problems
- Fever
- Sudden drop in body temperature

1. Discuss confusion and delirium

Causes of confusion (cont'd):

- Lack of oxygen
- Medications
- Infections
- Brain tumor
- Diseases or illness
- Loss of sleep
- Seizures

1. Discuss confusion and delirium

When caring for a resident with confusion:

- Do not leave a confused resident alone.
- Stay calm. Provide a quiet environment.
- Speak in a lower tone of voice. Speak clearly and slowly.
- Introduce yourself each time you see resident.
- Remind resident of location, name, and date.
- Explain what you are going to do using simple instructions.
- Be patient. Do not rush the resident.
- Talk about plans for the day. Keep a routine.
- Encourage the use of eyeglasses and hearing aids.

1. Discuss confusion and delirium

When caring for a resident with confusion (cont'd):

- Promote self-care and independence.
- Do not leave cleaning agents or personal care products where the resident can access them.
- Report observations to the nurse.

1. Discuss confusion and delirium

Causes of delirium include the following:

- Infections
- Disease
- Fluid imbalance
- Poor nutrition
- Drugs
- Alcohol

1. Discuss confusion and delirium

These are all symptoms of delirium:

- Agitation
- Anger
- Depression
- Irritability
- Disorientation
- Trouble focusing
- Problems with speech
- Changes in sensation and perception
- Changes in consciousness
- Decrease in short-term memory

1. Discuss confusion and delirium

REMEMBER:

It helps to be gentle and to keep one's voice low when communicating with someone who is confused or disoriented. Use the person's name and speak clearly, using simple sentences. Reduce distractions as much as possible.

2. Describe dementia and discuss Alzheimer's disease

Define the following terms:

cognition

the ability to think logically and clearly.

cognitive impairment

loss of ability to think logically and clearly.

dementia

the serious loss of mental abilities, such as thinking, remembering, reasoning, and communicating.

Alzheimer's disease

a progressive, incurable disease that causes tangled nerve fibers and protein deposits to form in the brain, which eventually cause dementia.

2. Describe dementia and discuss Alzheimer's disease

REMEMBER:

Dementia is *not* a normal part of aging.

2. Describe dementia and discuss Alzheimer's disease

The following are common causes of dementia:

- Alzheimer's disease
- Multi-infarct or vascular dementia (a series of strokes causing damage to the brain)
- Lewy Body disease
- Parkinson's disease
- Huntington's disease

2. Describe dementia and discuss Alzheimer's disease

Diagnosing dementia is a complicated process:

- Involves getting a patient's medical history and having a physical and neurological exam.
- Blood tests and imaging tests like CT or MRI scans may be ordered.
- Tests to trace brain wave activity (like EEGs) may be performed.
- Diagnosis of dementia helps rule out other possible diseases with similar symptoms.

Transparency 5-1: Facts about Alzheimer's Disease

- Alzheimer's disease is the most common cause of dementia in the elderly.
- As many as 5.5 million people in the U.S. are living with Alzheimer's disease.
- One in 10 people aged 65 and older has AD
- Women are more likely than men to have AD.
- African-Americans are about two times as likely to get AD as older whites, while Hispanics are about 1.5 times as likely.
- Risk increases with age, but it is not a normal part of aging.
- AD is progressive, degenerative, and irreversible.
- Tangled nerve fibers and protein deposits in brain cause dementia.
- Cause is currently unknown and diagnosis is difficult.
- Length of time from onset to death can range from 4 to 20 years.
- Each person will show different symptoms at different times.

2. Describe dementia and discuss Alzheimer's disease

NAs should remember these points when caring for residents with AD:

- Every person with Alzheimer's progresses differently, showing different symptoms at different times.
- Residents should be encouraged to do self-care and keep their minds and bodies active for as long as possible.

Transparency 5-2: Helpful Attitudes for Working with Residents with AD

- Do not take things personally.
- Be empathetic.
- Work with symptoms and behaviors noted.
- Work as a team.
- Be aware of the difficulties associated with caregiving.
- Work with family members.
- Remember the goals of the care plan.

3. List strategies for better communication with residents with Alzheimer's disease

Define the following term:

perseveration

the repetition of words, phrases, questions, or actions.

3. List strategies for better communication with residents with Alzheimer's disease

When communicating with a resident with AD:

- Approach from the front.
- Smile and look happy to see the resident. Be friendly.
- Determine how close the resident wants you to be.
- Communicate in a calm area with little background noise and distraction.
- Always identify yourself. Use the resident's name and continue to use the resident's name.
- Speak slowly, using a lower tone of voice.

3. List strategies for better communication with residents with Alzheimer's disease

When communicating with a resident with AD (cont'd):

- Repeat yourself, using the same words and phrases, as often as needed.
- Use signs, pictures, gestures, or written words to help communicate.
- Break complex tasks into smaller, simpler ones.

3. List strategies for better communication with residents with Alzheimer's disease

If resident is frightened or anxious

- Speak in a low, calm voice. Speak in a quiet area with few distractions.
- Try to see and hear yourself as residents might. Describe what you are going to do.
- Use simple words and short sentences.
- Check your body language. Make sure you are not tense or hurried.

3. List strategies for better communication with residents with Alzheimer's disease

If resident forgets or shows memory loss

- Repeat yourself using the same words. If a resident does not understand a word, try a different one. If resident perseverates, answer questions using the same words each time.
- Keep messages simple. Break complex tasks into smaller, simpler ones.

3. List strategies for better communication with residents with Alzheimer's disease

If resident has trouble finding words or names

- Suggest a word that sounds correct. Try not to correct a resident who uses an incorrect word.

If resident does not seem to understand basic instructions or questions

- Ask resident to repeat your words. Use short words and sentences. Allow time to answer.
- Use the communication methods that are effective.
- Watch for nonverbal cues. Observe body language.
- Use signs, pictures, gestures, or written words.

3. List strategies for better communication with residents with Alzheimer's disease

If resident wants to say something but cannot

- Encourage resident to point, gesture, or act it out.
- Offer comfort with a smile if resident is upset or try to distract her.

If resident does not remember how to perform basic tasks

- Break each activity into simple steps.

3. List strategies for better communication with residents with Alzheimer's disease

If resident insists on doing something that is unsafe or not allowed

- Limit the times you say “Don’t.” Redirect activities instead.

If resident hallucinates or is paranoid or accusing

- Try not to take it personally.
- Try to redirect behavior or ignore it.

3. List strategies for better communication with residents with Alzheimer's disease

If resident is depressed or lonely

- Take time one-on-one to ask how he is feeling. Listen to the response.
- Try to involve the resident in activities. Report signs of depression to the nurse.

If resident repeatedly asks to go home

- Ask the resident to tell her what his home was like and how he felt being there.
- Redirect to something he enjoys.
- Expect questions to continue and remain patient and gentle with responses.

3. List strategies for better communication with residents with Alzheimer's disease

If resident is verbally abusive or uses bad language

- Remember it is the dementia speaking, not the person. Try to ignore the language. Redirect attention.

If resident has lost most verbal skills

- Use nonverbal skills, such as touch, smiles and laughter.
- Use signs, labels and gestures.
- Assume people can understand more than they can express.

4. List and describe interventions for problems with common activities of daily living (ADLs)

NAs working with residents with AD should remember these three general principles for providing care:

- Develop a routine and stick to it.
- Promote self-care.
- Take good care of yourself, both mentally and physically.

Handout 5-1: Interventions for ADLs

For problems with bathing

- Schedule bathing when resident is least agitated.
- Give resident supplies before bathing to serve as visual aid.
- Take a walk with resident down the hall and stop at tub or shower room.
- Make sure bathroom is well-lit and at a comfortable temperature.
- Provide privacy.
- Be calm and quiet. Keep the process simple.
- Be sensitive when discussing bathing.
- Give resident washcloth to hold during the bath.
- Ensure safety by using nonslip mats, tub seats, and hand-holds.
- Be flexible about when to bathe. Understand if resident does not want to bathe.
- Be relaxed. Offer encouragement and praise.

Handout 5-1: Interventions for ADLs (cont'd)

- Let the resident do as much as possible for himself.
- Check the skin for signs of irritation.

For problems with grooming and dressing

- Help with grooming.
- Avoid delays or interruptions.
- Show resident the clothing to put on.
- Provide privacy.
- Encourage resident to pick out clothes to wear. Lay out clothes in order to be put on.
- Break task down into simple steps. Do not rush the resident.
- Use a friendly, calm voice when speaking. Praise and encourage.

Handout 5-1: Interventions for ADLs (cont'd)

For problems with toileting

- Encourage fluids
- Mark bathroom with sign or picture.
- Make sure there is enough light in the bathroom and on the way there.
- Note when resident is incontinent. Check him every 30 minutes. Take resident to bathroom before bathroom time.
- Observe toilet patterns for two to three nights if the resident is incontinent during night.
- Take resident to bathroom after drinking fluids. Make sure resident urinates before getting off toilet.
- Take resident to bathroom before and after meals and before bed.
- Put lids on trash cans, wastebaskets, or other containers if resident urinates or defecates in them.
- Be professional when cleaning after episodes of incontinence.

Handout 5-1: Interventions for ADLs (cont'd)

For problems with nutrition

- Encourage nutritious food.
- Have meals at consistent times each day. Food should look and smell appetizing.
- Make sure there is proper lighting.
- Keep noise and distractions low
- Keep the task of eating simple. Finger foods are easier to eat.
- Do not serve steaming or very hot foods or drinks.
- Use plain plates without a pattern or color. Use a simple place setting with a single utensil. Remove other items from the table.
- Put only one item of food on the plate at a time.
- Give simple, clear instructions for eating or using utensils. Place a spoon to the lips. Ask the resident to open his mouth.

Handout 5-1: Interventions for ADLs (cont'd)

- Guide the resident through the meal with simple instructions. Offer regular drinks to avoid dehydration.
- Use adaptive equipment as needed.
- Feed resident slowly, giving small pieces of food.
- Make mealtimes simple and relaxed.
- Give resident time to swallow each bite.
- Seat residents with others to encourage socializing.
- Observe for eating and swallowing problems. Observe and report changes or problems. Monitor weight frequently.

Handout 5-1: Interventions for ADLs (cont'd)

To promote physical health

- Prevent infections and follow Standard Precautions.
- Observe the resident's physical health and report any potential problems
- Help the resident wash their hands frequently.
- Give careful skin care to prevent pressure injuries.
- Watch for signs of pain. Report possible signs of pain to the nurse.
- Maintain daily exercise routine.

To promote mental and emotional health

- Maintain self-esteem by encouraging independence.
- Share in enjoyable activities, looking at pictures, talking and reminiscing.
- Reward positive and independent behavior with smiles and warm touches.

5. List and describe interventions for common difficult behaviors related to Alzheimer's disease

Define the following terms:

sundowning

becoming restless and agitated in the late afternoon, evening, or night.

catastrophic reaction

reacting to something in an unreasonable, exaggerated way.

pacing

walking back and forth in the same area.

5. List and describe interventions for common difficult behaviors related to Alzheimer's disease

Define the following terms:

wandering

walking aimlessly around the facility or facility grounds.

elope

in medicine, when a person with Alzheimer's disease wanders away from a protected area and does not return.

hallucinations

false or distorted sensory perceptions.

5. List and describe interventions for common difficult behaviors related to Alzheimer's disease

Define the following terms:

delusions

persistent false beliefs.

rummaging

Going through drawers, closets or personal items that belong to oneself or others.

hoarding

collecting and putting things away in a guarded way.

Handout 5-2: Difficult Behaviors and Management

Agitation

Remove triggers, keep routine, reduce noise, focus on familiar activity, remain calm, and soothe.

Sundowning

Avoid stressful situations, limit activities and appointments, play soft music, set a bedtime routine, plan a calming activity, limit caffeine, provide snacks, give a back massage, distract, and maintain a daily exercise program.

Catastrophic reactions

Avoid triggers such as fatigue, changes, overstimulation, difficult choices/tasks, pain, hunger, or need to use the toilet. Remove triggers and distract.

Violent behavior

Call for help, Block blows, never hit back, step out of reach, do not leave the resident alone, remove triggers, and use the same calming techniques as for agitation and sundowning.

Handout 5-2: Difficult Behaviors and Management (cont'd)

Pacing and wandering

Causes: restlessness, hunger, disorientation, incontinence or need to use the toilet, constipation, pain, forgetting how or where to sit down, too much napping, need for exercise.

Remove causes, give snacks, encourage exercise, maintain toileting schedule, let resident pace in safe place, redirect attention, and mark rooms with signs or pictures such as stop signs.

Hallucinations or delusions

Ignore if harmless, reassure, do not argue, and be calm.

Depression

Causes: loss of independence, inability to cope, feelings of failure and fear, facing an incurable illness, or a chemical imbalance.

Report signs, observe for triggers that cause changes in mood, encourage independence, talk about moods and feelings, and encourage social interaction.

Handout 5-2: Difficult Behaviors and Management (cont'd)

Perseveration or Repetitive Phrasing

Respond with patience, do not stop behavior, and answer questions each time, using the same words.

Disruptiveness

Gain resident's attention, be calm, direct to a private area, ask about behavior, notice and praise improvements, tell resident about any changes, encourage the resident to join in activities, help the resident find ways to cope, and focus on activities the resident may still be able to do.

Inappropriate Social Behavior

Do not take it personally, stay calm, reassure, find out cause, direct to private area, respond positively to appropriate behavior, and report abuse to nurse.

Handout 5-2: Difficult Behaviors and Management (cont'd)

Inappropriate Sexual Behavior

Stay calm and be reassuring, try to determine the cause of the problem (is the behavior intentional?), direct to a private area, and consider other ways to provide physical stimulation.

Hoarding and rummaging

Label belongings, place a label or symbol on door, do not tell others that person is stealing, prepare the family, ask the family to report unfamiliar items, and provide a rummage drawer.

Sleep disturbances

Make sure resident gets moderate exercise/activity throughout the day. Allow the resident to spend time in natural sunlight if possible. Reduce light and noise during nighttime hours. Discourage sleeping during the day.

5. List and describe interventions for common difficult behaviors related to Alzheimer's disease

REMEMBER:

Residents with AD may try to elope or leave a facility unsupervised and unnoticed. If a resident with AD elopes the NA must inform the nurse right away. The earlier a search is begun, the more likely the resident is to be found nearby and safe.

People with AD may be at a higher risk for abuse. One reason for this is that caring for someone with AD is very demanding, physically and psychologically. NAs should take good care of themselves to ensure they are able to provide the best possible care.

6. Describe creative therapies for residents with Alzheimer's disease

Define the following term:

validating

giving value to or approving.

6. Describe creative therapies for residents with Alzheimer's disease

Three creative therapies may be useful in working with residents with Alzheimer's disease:

- Validation therapy allows residents to believe they live in the past or in imaginary circumstances.
- Reminiscence therapy encourages residents to remember and talk about the past.
- Activity therapy uses activities that the resident enjoys to prevent boredom and frustration and to promote self-esteem.

6. Describe creative therapies for residents with Alzheimer's disease

REMEMBER:

Music therapy has been used successfully with people who have AD. Music is a form of sensory stimulation. Hearing familiar songs may cause a positive response in people with dementia who do not respond well to other treatments. Music therapy may be utilized to accomplish specific goals such as managing stress, improving mood, or enhancing cognition. The NA can help by observing the resident's response and playing the music she enjoys.

6. Describe creative therapies for residents with Alzheimer's disease

Think about these questions:

At which stages of Alzheimer's disease might each of these therapies be useful?

What are the benefits of these therapies?



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